

D14 – Office Visit Note

M.K. Osei, MD | Internal Medicine | 14 Feb 2023

Patient: Caduceus, Opus J. MRN: 004-7703 DOB: 14 Mar 1956 Allergies: NKDA

OFFICE VISIT NOTE

Patient: Opus J. Caduceus | MRN: 004-7703

Date: 02/14/2023 | Provider: M.K. Osei, MD

Visit Type: Established Patient / Follow-Up

Practice: Norfolk Veterans Medical Associates, Norfolk VA

CHIEF COMPLAINT: Sleep medicine follow-up. Persistent fatigue and daytime somnolence despite

HISTORY OF PRESENT ILLNESS:

66-year-old retired USN Senior Chief Petty Officer presenting for routine chronic condition management with primary concern of persistent daytime fatigue and somnolence. Recently seen by sleep medicine (Dr. T. Adeyemi, Norfolk Sleep Center) on 28 Jan 2023. CPAP device download confirmed non-adherence with average use less than 2 hours per night over the past 12 months. Patient describes mask intolerance and positional discomfort as primary barriers despite multiple mask fittings. Sleep medicine has formally discontinued CPAP. Patient declined oral appliance therapy when offered. Discussed alternatives. All other chronic conditions stable per patient.

CURRENT MEDICATIONS:

- Omeprazole 20 mg PO daily
- Metoprolol succinate 25 mg PO daily
- Rivaroxaban 20 mg PO daily
- Atorvastatin 40 mg PO daily
- Aspirin 81 mg PO daily
- Isosorbide mononitrate 30 mg PO daily
- Tamsulosin 0.4 mg PO daily
- Topiramate 25 mg PO daily
- Naproxen 500 mg PO BID PRN
- Diclofenac gel PRN (knees)
- Flunisolide nasal spray PRN
- Lidocaine 5% patch PRN (knees)
- CPAP DISCONTINUED 28 Jan 2023 per sleep medicine

PHYSICAL EXAMINATION:

Vitals: HR 72 | BP 138/82 | RR 14 | SpO2 96% RA | Wt 93 kg

General: Mildly fatigued appearing. NAD.

HEENT: Mallampati III. Mild oropharyngeal crowding. Patent nasal passages.

CV: Rate-controlled, regular. No murmurs.

Lungs: Clear bilaterally.

Neuro: At baseline. No focal deficits. Gait stable.

MSK: Bilateral knee tenderness R > L. Crepitus with passive ROM. No effusion.

ASSESSMENT & PLAN:

1. Obstructive Sleep Apnea, CPAP non-adherent - sleep medicine follow-up (Dr. T. Adeyemi, Norfolk Sleep Center, 28 Jan 2023): CPAP non-adherent (average use <2 hours per night over the past 12 months per device download), mask intolerance and positional discomfort persistent despite mask refittings. CPAP DISCONTINUED 28 Jan 2023 per sleep medicine recommendation. Residual daytime hypersomnolence formally documented - Epworth Sleepiness Scale

14/24. Oral appliance therapy discussed; patient declines. Conservative measures counseled (sleep hygiene, weight management, avoidance of supine position, alcohol avoidance in evening). Sleep medicine follow-up in 6 months.

2. Cerebrovascular / Moyamoya vasculopathy - MRI brain (15 Apr 2022, Norfolk Radiology) findings consistent with Moyamoya vasculopathy per Dr. Okafor neurology read. Neurology managing. No acute changes since diagnosis. No further TIA-like episodes. Driving restored per neurology after stable interval.
3. Ischemic heart disease, stable angina - stable on isosorbide mononitrate, ASA, atorvastatin. Cardiology follow-up current with Dr. Mensah, Norfolk Cardiology Associates.
4. Atrial fibrillation - rate-controlled on metoprolol. Anticoagulated on rivaroxaban. Stable.
5. GERD - stable post Nissen x2. Continue omeprazole 20 mg daily.
6. BPH - stable on tamsulosin. PSA surveillance current.
7. Migraine headaches - stable on topiramate. Approximately q6-8 weeks pattern unchanged. No change.
8. PTSD - service-connected. Managed by VA Mental Health Norfolk. Stable per patient.
9. Bilateral knee pain - chronic, R > L. Diclofenac gel and lidocaine patch ongoing as needed. No new orthopedic referral at this time. Will reassess if functional decline.

Labs/Orders: BMP, lipid panel, HbA1c

Follow-up: Sleep medicine 6 months. Continue chronic condition management. PCP RTC 3 months.

Electronically signed by: M.K. Osei, MD

Date: 02/14/2023 11:20

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